

PARTAMOL EFF. EFFERVESCENT TABLETS 500MG

1. Name of the medicinal product

PARTAMOL EFF. EFFERVESCENT TABLETS 500MG

2. Special notice and recommendation

Keep out of reach of children

Read the package insert carefully before use

3. Composition

Each effervescent tablet contains:

Paracetamol 500 mg

4. Pharmaceutical form

Effervescent tablet.

White to off-white, round tablet, flat and plain on both sides. When dissolved in water, the tablet effervesces to produce a colorless solution with characteristic aroma.

5. Indications

Paracetamol is a mild analgesic and antipyretic, and is recommended for the treatment of most painful and febrile conditions, for example, headache including migraine and tension headaches, toothache, backache, rheumatic and muscle pains, dysmenorrhoea, sore throat, and for relieving the fever, aches and pains of colds and flu.

6. Administration and dosage

Administration

Oral administration only.

Dosage

Adults, the elderly, and children aged 12 years and over:

1-2 tablets in at least half a glass of water, up to 4 times daily every 4 to 6 hours as required, up to 8 tablets daily.

Children:

Aged 9-11 years: 1 tablet dissolved in in at least half a glass of water. Maximum daily dose: 60mg/kg paracetamol to be administered in divided doses throughout the 24-hour period. Doses of paracetamol should not be given more frequently than every 4 hours, and not more than 4 doses should be given in any 24 hour period.

Not recommended for children under the age of 9 years.

7. Contraindications

Hypersensitivity to paracetamol or any of the other constituents.

8. Special warnings and precautions for use

This preparation contains PARACETAMOL.

Do not take any other paracetamol containing medicines at the same time.

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

- Underlying liver disease increases the risk of paracetamol related liver damage. Patients who have been diagnosed with liver or kidney impairment must seek medical advice before taking this medication.
- Do not exceed the stated dose.
- Patients should be advised to consult their doctor if their headaches become persistent.
- Patients should be advised to consult a doctor if they suffer from non-serious arthritis and need to take painkillers every day.
- Patients should be advised to consult a doctor if they are underweight or malnourished before use.
- Patients should be advised to consult a doctor if they regularly drink alcohol before use.
- Patients should be advised to consult a doctor before use if they have a severe infection as this may increase the risk of metabolic acidosis. Signs of metabolic acidosis include: shallow, rapid, difficult breathing, feeling sick (nausea), being sick (vomiting), loss of appetite.
- Each effervescent tablet contains 329 mg of sodium per tablet. To be taken into consideration by patients on a controlled sodium diet.
- If symptoms persist, medical advice must be sought.
- Keep out of the sight and reach of children.
- This product contains aspartame. Unsuitable for phenylketonurics.

9. Pregnancy and lactation

Epidemiological studies on neurodevelopment in children exposed to paracetamol in utero show inconclusive results. If clinically needed, paracetamol can be used during pregnancy if clinically needed, however, as with any medicine it should be used at the lowest effective dose for the shortest possible time.

Paracetamol is excreted in breast milk but not in a clinically significant amount in recommended dosages. Available published data do not contraindicate breast feeding.

10. Effects on ability to drive and use machines

None.

11. Drug interactions

The speed of absorption of paracetamol may be increased by metoclopramide or domperidone and absorption reduced by colestyramine. The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular daily use of paracetamol with increased risk of bleeding; occasional doses have no significant effect.

12. Adverse reactions

Adverse events of paracetamol from historical clinical trial data are both infrequent and from small patient exposure.

Accordingly, events reported from extensive post-marketing experience at therapeutic/labelled dose and considered attributable are tabulated below by system class and frequency. The following convention has been utilised for the classification of the undesirable effects: very common ($\geq 1/10$), common ($\geq 1/100$ to $< 1/10$), uncommon ($\geq 1/1000$ to $< 1/100$), rare ($\geq 1/10,000$ to $< 1/1000$) and very rare ($< 1/10,000$), not known (cannot be estimated from available data).

Adverse event frequencies have been estimated from spontaneous reports received through post-marketing data.

Blood and lymphatic system disorders

Very rare: Thrombocytopenia, agranulocytosis.

Immune system disorders

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

Respiratory, thoracic and mediastinal disorders

Very rare: Bronchospasm *

Hepatobiliary disorders

Very rare: Hepatic dysfunction

* There have been cases of bronchospasm with paracetamol, but these are more likely in asthmatics sensitive to aspirin or other NSAIDs.

13. Overdosage and management

Liver damage is possible in adults who have taken 10 g or more of paracetamol. Ingestion of 5 g or more of paracetamol may lead to liver damage if the patient has risk factors (see below).

Risk Factors:

If the patient

- Is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St John's Wort or other drugs that induce liver enzymes.

Or

- Regularly consumes ethanol in excess of recommended amounts.

Or

- Is likely to be glutathione deplete e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia.

Symptoms

Symptoms of paracetamol overdose in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.

Management

Immediate treatment is essential in the management of paracetamol overdose. Despite a lack of significant early symptoms, patients should be referred to hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines, see BNF overdose section.

Treatment with activated charcoal should be considered if the overdose has been taken within 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable). Treatment with N-acetylcysteine may be used up to 24 hours after ingestion of paracetamol, however, the maximum protective effect is obtained up to 8 hours post-ingestion. The effectiveness of the antidote declines sharply after this time. If required the patient should be given intravenous N- acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, oral methionine may be a suitable alternative for remote areas, outside hospital. Management of patients who present with serious hepatic dysfunction beyond 24h from ingestion should be discussed with the NPIS or a liver unit.

High doses of sodium bicarbonate may be expected to induce gastrointestinal symptoms including belching and nausea. In addition, high doses of sodium bicarbonate may cause hypernatraemia; electrolytes should be monitored and patients managed accordingly.

14. Pharmacodynamic properties

Pharmacotherapeutic group: Other analgesics and antipyretics; anilides.

ATC code: N02BE01.

Paracetamol is a well-established analgesic.

15. Pharmacokinetic properties

Paracetamol is rapidly and almost completely absorbed from the gastrointestinal tract. Concentration of the drug in plasma reaches a peak in 30 - 60 minutes and the plasma half-life is 1 - 4 hours.

Paracetamol is relatively uniformly distributed throughout most body fluids and exhibits variable protein binding. Excretion is almost exclusively renal in the form of conjugated metabolites.

16. Packaging

Strip of 4 tablets. Box of 4 strips.

17. Storage condition, shelf-life

Protect from light and moisture. Do not store above 30°C.

The expiry date of this pack is printed on the box. Do not use this pack after this date.

18. Name, address of manufacturer



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Date of revision: September 20, 2024
Text font: Arimo; Size: 7

PI200924